

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION

Workers' Compensation and Rehabilitation Act 2003 (Qld)

Matter No: WC/2024/227

BETWEEN:

CORY LEA SHEPHERD

(Appellant)

and

WORKERS' COMPENSATION REGULATOR

(Respondent)

AMENDED STATEMENT OF FACTS AND CONTENTIONS

(Filed pursuant to leave of the Commission)

PART A — IDENTIFYING INFORMATION

Appellant:	Cory Lea Shepherd
Employer:	State of Queensland (Queensland Health) – Logan Hospital Switchboard
Role:	Administration Officer (OO3)
Injury:	Major Depressive Disorder with Anxious Distress (DSM-5 296.23)
Date of Injury Onset:	18 June 2024

PART B — STATEMENT OF MATERIAL FACTS

1. The Worker and the Clean Baseline

- 1.1 At all material times, the Appellant was employed by the Employer in the Switchboard Department at Logan Hospital, coordinating life-safety emergency clinical codes (Code Blue, MET calls).
- 1.2 The Appellant possessed a clean psychiatric baseline prior to the workplace conduct. The Employer's own clinical records from 16 November 2023 expressly confirm: "No psychological illness such as depression/psychosis" (**Admitted Fact: Form 24, Para 34**).

1.3 On 18 June 2024, the Appellant suffered the onset of a psychiatric injury, subsequently diagnosed by Dr Ravikumar Krishnaiah as Major Depressive Disorder (**Admitted Fact: Form 24, Para 38**). Dr Krishnaiah explicitly warned that premature workplace exposure would likely result in clinical deterioration (**Admitted Fact: Form 24, Para 39**).

1.4 The Appellant's treating General Practitioner, Dr Hawes, recorded the contemporaneous mechanism of injury as: "ongoing breaking of workplace rules by bosses, victimizing him" (**Admitted Fact: Form 24, Para 36**).

2. The Causative Stressors

The Appellant's psychological injury was caused by a composite course of management conduct occurring between approximately June 2023 and June 2024:

Stressor 1: A Hostile Course of Management Conduct, Reprisal, and Suppression of Rights

- (a) **Dereliction of Clinical Governance:** Throughout 2023 and 2024, the Appellant's Line Manager, Ms. Chloe Taylor, maintained an erratic physical presence and imposed unassessed unilateral directives (extra administrative checks/paging) without consultation. These arbitrary directives bottlenecked emergency workflows and directly caused verifiable delays to the communication of urgent pathology results to clinical staff. The Appellant's responsible attempts to mitigate these clinical risks triggered the manager's subsequent hostility.
- (b) **Unilateral Destruction of WHS Records:** On 6 June 2023, the Appellant recorded a critical operational instruction in the communication book regarding the necessity of updating medical on-call contact numbers. Ms. Taylor unilaterally removed and destroyed this entry. When questioned, Ms. Taylor subjected the Appellant to a hostile and verbally aggressive reprimand. Ms. Taylor later admitted the destruction in writing: "I took it out last week" (**Admitted Fact: Form 24, Para 14**).
- (c) **Refusal to Investigate WHS & Fatigue Complaints:** On 7 August 2023, the Appellant submitted a formal grievance regarding Ms. Taylor's conduct, specifically raising WHS concerns regarding unsafe time rostering practices and fatigue management risks. Ms. Tammy Reese (Director) dismissed the complaint the same day without formal investigation and directed the Appellant to continue reporting directly to Ms. Taylor, thereby endorsing the unsafe rostering practices, ignoring the clinical fatigue risks, and normalizing the conduct (**Admitted Fact: Form 24, Para 5**).
- (d) **Disparate Treatment & Statutory Leave Obstruction (COVID):** Between 20 February and 1 March 2024, Ms. Taylor forced the actively ill Appellant to self-administer COVID-19 Special Pandemic Leave via 'MyHR'—a facilitation she routinely exercised directly for other staff. The system server logs timestamp the Appellant's successful upload at exactly 11:41:27 AM on 20 February 2024. Despite this objective system log, Ms. Taylor capriciously declined the application twice, advancing factually erroneous claims contradicted by objective server logs that

evidence was "not attached," before approving it using the identical original evidence. Forcing an incapacitated employee to repeatedly navigate an administrative portal to overcome factually false denials constitutes unreasonable management action.

- (e) **Public Interest Disclosure (PID):** On 13 May 2024, the Appellant lodged a corrupt conduct complaint regarding clinical risks. The Ethical Standards Unit formally determined this constituted a Public Interest Disclosure (**Admitted Fact: Form 24, Para 20**).
- (f) **Immediate Reprisal:** Within 48 hours of the PID being lodged, Ms. Reese directed the Appellant to retract a routine workplace email. A highly comparable email sent by Ms. Taylor attracted no such discipline (**Admitted Fact: Form 24, Para 21**).
- (g) **Suppression of Industrial Representation During Roster Disputes:** In April 2023, the Appellant formally expressed interest in assuming the role of union delegate (**Admitted Fact: Form 24, Para 18**). In direct contravention of Union Encouragement Policy QH-POL-248 (**Admitted Fact: Form 24, Para 17**), the Employer actively suppressed this appointment for 13 months. Crucially, this deliberate administrative inaction left the department operating without local delegate representation precisely during a period when the Appellant was actively attempting to dispute unsafe time rostering practices and FRMS fatigue breaches, effectively stripping the Appellant of statutory WHS protections.

Stressor 2: Systemic Failure to Discharge Remuneration Obligations

- (a) **Persistent Payroll Failures:** Between February and April 2024, the Employer systemically failed to pay the Appellant his correct statutory entitlements, resulting in a documented 42% pay disparity.
- (b) **Unreasonable Delay:** On 3 May 2024, Queensland Health Payroll explicitly identified the errors and directed Ms. Taylor to process corrections "IMMEDIATELY" (**Admitted Fact: Form 24, Para 40**). Ms. Taylor failed to action this urgent directive until 28 May 2024—an admitted 25-day delay (**Admitted Fact: Form 24, Para 41**).

Stressor 3: Admitted Breaches of Statutory Fatigue Management

- (a) **Mathematical Reality of the WHS Breach:** On 17-18 March 2024, the Employer rostered the Appellant to work consecutive shifts separated by a mere 7-hour break (**Admitted Fact: Form 24, Para 1**). Factoring in the Appellant's established commute, the Appellant was subjected to 16 hours of wakefulness/work and 4 hours of high-risk commuting, leaving approximately 4 hours of actual sleep.
- (b) **Flagrant Bypass of the 'Neville' Safety Mandate Despite Prior Warnings:** The Award and the FRMS mandate a minimum 10-hour rest break (**Admitted Fact: Form 24, Para 3**). The Queensland Health FRMS is a strict, non-delegable clinical safety framework implemented following the 2006 Ombudsman 'Neville Report'. Despite the Appellant having previously raised explicit formal complaints to management

regarding these exact fatigue risks, the Employer proceeded to roster the Appellant on a 7-hour break. Operating under extreme, employer-induced exhaustion (4 hours sleep), the Appellant was required to manage life-safety emergency codes and doctor pages via the SPOK system. Forcing an exhausted operator to control emergency clinical coordination while actively ignoring prior WHS fatigue warnings is objectively reckless.

- (c) **Unjustified Administrative Detriment:** When the Appellant predictably required recovery time from this extreme exhaustion, Ms. Taylor engaged in a fundamentally unreasonable exercise of management authority. She misapplied a superseded 2020 casual agreement to deny statutory paid fatigue leave, compelling the Appellant to exhaust his own accrued personal leave on 19 March 2024. This effectively shifted the financial detriment of the Employer's WHS breach onto the injured worker.
- (d) **Admitted Unreasonableness:** The Regulator's own Independent Review Office (IRO) formally found that this 7-hour rest break constituted unreasonable management action (**Admitted Fact: Form 24, Para 37**).

3. Subsequent Aggravating Conduct (Post-Injury Unreasonableness)

- (a) On 12 and 16 July 2024, while incapacitated by certified psychological leave, the Employer scheduled meetings regarding his complaint.
- (b) On 8 October 2024, the Employer terminated the Appellant under "abandonment" provisions despite holding continuous medical certificates. The Appellant was subsequently reinstated.
- (c) During proceedings, the Respondent unilaterally obtained the Appellant's primary medical records without issuing the mandatory Form 29 Notice (Rule 64C), depriving the Appellant of statutory objection rights (**Admitted Fact: Form 24, Para 25**).

PART C — LEGAL CONTENTIONS

Contention 1: The Injury is Compensable (s.32(1) WCRA)

The Appellant sustained a "personal injury" arising out of employment.

Contention 2: The Section 32(5)(a) Exclusion Fails Entirely

The Respondent bears the onus under *Prizeman v Q-Comp*.

- (a) **Global Evaluation (Delaney):** The clinical governance dereliction, WHS records destruction, 25-day AVAC delay, and PID retaliation form a composite, hostile course of conduct.
- (b) **Single Unreasonable Stressor (Mahaffey):** The Employer's admitted breach of the 7-hour rest break independently defeats the statutory defense.

Contention 3: Evidentiary Contradiction

The Respondent's pleading that "all management action was reasonable" is in direct evidentiary contradiction with its own IRO finding.

PART D — ORDERS SOUGHT

1. Appeal allowed; Decision set aside and replaced with claim acceptance.
2. A declaration that the Appellant's psychological injury is a compensable injury.
3. Costs reserved.
4. Such further or other orders as the Commission considers appropriate.

FILED BY:

Cory Lea Shepherd

Appellant (Self-Represented)

DATE: 7 April 2026